

I: Then let's start with the actual questions.

I: Let's start with familiarity with AI.

I: How familiar are you with AI technologies in general?

P: I know the basic ones, like the LLMs, like the ChatGPTs, the Claudes, the Google AI overviews, those ones.

P: And recently at work, we've also started using Medendo, which is also like a transcription that turns it into a report, which is also like supported by AI, so that's the one I'm also familiar with.

I: Okay, and then also like gen AI which is like online and stuff like that.

I: Yeah, so your organization has already introduced some AI tools as well.

P: Yeah, it's like very much a pilot, like in the beginning stages.

I: So is it an actual pilot or is it still a pilot?

P: It's still a pilot.

I: Yeah.

P: It hasn't gone throughout the entire organization.

I: Yeah.

I: And you've only introduced Medendo.

P: Yeah.

I: Okay.

I: And apart from this, you're not using any AI during work, right?

P: No.

I: Okay.

I: But you are familiar in some ways.

P: Yeah.

I: Can you also give a score on this?

I: How familiar you think you are, from one being very low familiarity and ten being very high?

P: Seven.

I: Okay, so you said you're using Medendo, right?

I: Did you also disclose this with your clients that you're using this?

P: Yeah.

I: Okay, then we can move on to the next question.

I: This is about diagnostics and assessment use.

I: What are your thoughts on AI being used in diagnostic or assessment processes?

P: So obviously when we started using Medendo, this was a really big question of like, what kind of role is Medendo going to have in the process?

P: The main conclusion we all had, and something I also strongly agree with, is that Medendo, it makes sense to use it in terms of like, okay, I need to write a report.

P: Can I write down my thoughts and all my annotations and can they create a report from that?

P: But the main conclusion is that a person should still be behind actually writing down what the conclusion is.

P: It's just like making from notes to making it a full report.

P: That's something I feel like, okay, an AI could be used in that way.

P: But in terms of an assessment, I don't think, I don't even think we have the proper technology that they can make an accurate assessment with everything.

P: Yeah, I don't think the technology is even there yet.

I: For AI systems to do this, or in general? Or AI systems?

P: Yeah.

I: So how does the normal diagnostics and assessment work in your day to day?

P: So for example, if somebody comes to us for an autism assessment, they come in for an intake from like one and a half to two hours where we ask a bunch of questions.

P: And then we start with the autism assessment, which is usually up to like four hours of an interview, where we do the autism DSM-5 interview, and based on that interview we also score if they adhere to the criteria.

P: And after that interview we also have additional interviews where we talk to, we call it developmental anamnesis and a heteroanamnesis, where we talk to a person who knows them really well in the present, and then we also talk to somebody who knew them really well in the past, so a mother or father.

P: Then we combine all of that information and we create one report.

P: And that report will decide if the person has or doesn't have autism, or if there's something else.

P: And the part that Medendo now does is just that we have those interviews and then Medendo creates a transcript of it and it creates a report of the transcript, but it's just not making any conclusions.

P: It's also not really deciding how severely somebody is struggling with something.

P: If they're like clinically suffering or under the clinical baseline, it's also not really saying anything about the complaint in and of itself.

P: It's just very much just making a full sentence from the notes that we're writing or from the transcript.

I: Yeah.

I: So it's actually transcribing the whole process, right?

I: And then writing it in a report, but you say it's not actually drawing any conclusions.

P: Yeah.

I: But in general, this diagnosing, is it an objective thing?

I: Because I feel like there is some subjectivity to this as well, right?

P: Yeah.

P: Well, it's funny you mentioned that, because it's a thing that we've been kind of checking with the Medendo reports.

P: Because in the beginning it was more suggestible, it was actually kind of leaning towards, for example, if somebody was describing their depression or their depressive symptoms, Medendo would sometimes write a report where it would almost kind of state that we're talking about a depression, while not taking into consideration that you wouldn't call the pattern of these symptoms a depression, you would call it something else, maybe like an autistic burnout.

P: That was just an example, but Medendo wouldn't take that into account and it would be quite suggestible reports.

P: So now they're kind of looking at how they can take that away or minimize it as much as possible.

P: And also whenever we have these reports, I never just copy paste them.

P: I kind of go through it again and I delete a lot and I edit a lot.

P: And the things that I do delete are sentences that I find to be too suggestible, or I find the nuance is lacking from it.

P: Yeah.

P: And I do think that the bias is always there.

P: So I think you should always edit it and always check it afterwards.

P: Because it's always going to be a bit suggestive.

I: Yeah.

I: So this regarding intake mostly.

I: So have you considered using AI for preliminary screening?

P: Yeah.

P: I mean, now we kind of have like questionnaires and those are also good screening tools.

P: I wouldn't necessarily know what the additional benefit would be of replacing that with AI, because the questionnaires now also do a really good job of screening, like are there a lot of trauma symptoms, is there really low mood, are we seeing somebody on the brink of a psychosis for example? Like there are some really good questionnaires for that, or like instrumental tools right now.

I: Yeah.

I: Okay.

I: And then summarizing patient information, so that's kind of what you're doing, right?

I: So do you only use it during intakes or also in general sessions?

P: Well, we can use it in treatment as well, then it'll also write a little report for what you did in the treatment.

P: But I've done it and it's fine, but I never really use the reports, because I noticed that the things that I find more important, or the things that I would want to write down, the report doesn't really find those things important.

P: So I usually just make my own, but it doesn't take too much time to just write my own report after treatment sessions.

P: But I do think for assessments they're quite handy.

I: Why is it more handy in those assessments?

P: I think it's also more handy because when you write a report you're basically also doing, a large part of that is already like transcribing it. And it's more about the factual information, while treatment is a little bit more about like how did the client really experience this, what did,

P: How were they also in the room?

P: What did they notice when they were there?

P: How did they react to the information I was telling them?

P: Or how was it for them to tell something?

P: And you notice so much more that I think a transcription is not going to catch.

I: Yeah, so maybe in the intake or screening part, essentially the first session, there's more objectivity, right? So there's just like measurements of actual facts, like how many days has this person struggled with sleep or whatever. And then in normal sessions you would have these things come up as well, but those are not necessarily the parts you find important for the report.

P: Yeah, maybe so. Yeah.

I: Okay, makes sense. Then let's move on. So this is about monitoring and progress tracking. How do you feel about the role of AI in tracking patient progress over time? What would that look like, how would AI track it?

P: In terms of like they would just ask,

I: Questions? Yeah, so I don't want to be suggestive, because I maybe have more knowledge on what it could do, but I also think it's important for me to figure out if knowledge about some opportunities doesn't exist.

P: Yeah, yeah. You know, I was thinking about what you mentioned earlier about AI being used as a screening tool, and if I can come back to that for a second, I did think about it later, like now everybody gets like five standard questionnaires and they all have to fill them in, and a person who has a regular mood also has to do the mood questionnaire.

P: I feel like if you do have an AI who's there and can kind of, based on the answers that they're giving, choose or pick, like okay, we should focus, this is a client where we should focus on psychosis or more like personality stuff, as opposed to this one which is a little bit more light and it's more like depression or anxiety, then I can imagine AI being a good tool in screening: which

screening tools should we even use, based on the answers that they're giving or based on an intake questionnaire they've filled out?

I: And then yeah, because now you're just giving a lot of screening tools, like basically everything, fill this all out, and maybe someone with anxiety has to fill out some depression questionnaire or something a bit more unrelated. But then there might be some opportunity to get a pre-screening where you only get screened for things that are relevant for you.

P: Yeah, in the sense that it's not bad if somebody has a regular mood and we still screen on mood, because it's also good for us to know that something is not there. But it could be more effective if somebody screens really high on trauma, that apart from the standard questionnaires, the AI could already kind of pinpoint, okay, there is this level of traumatic symptoms, we should also do these additional ones.

P: And that's something that we don't necessarily do in the preliminary screening, or we do it later on in the intake. But maybe, if you were to only think of the path of how can I get the most information in the quickest amount of time, then I think that would make sense if an AI tool was used in that regard.

P: But then if I'm also thinking about monitoring, I'm curious to know how would AI have access to the progress?

P: And that could be like, again, transcripts that they're giving the AI, which they are tracking and they can kind of see if something is getting better or worse. Or the clients or patients do a questionnaire which they continuously track, but then it wouldn't really be AI, because any other data tool could do that basically.

P: I don't know. The transcript makes sense that an AI could do that, but I don't know, I'm not sure if I can see the added benefit of an AI tracking progress, because I also feel like the psychologist is also there to track it.

I: Yeah, because currently what are you doing for monitoring and tracking?

P: Well, I mean, a huge part of it, it sounds silly, but you just do an evaluation appointment.

P: You do like 10 sessions and then you just evaluate with the client, and you have your own observations and the client themselves can also report how they experienced it, if things got better or got worse.

P: In addition to that, you also have end of session questionnaires that you do just to kind of see, compared to the baseline from a couple of months ago, what changed and what didn't.

P: And based on those numbers you yourself can make a conclusion or statement.

I: Yeah, so this is after 10 sessions, I mean for example if you're doing CBT that's one thing, if you're doing schema therapy that takes longer. And you just do it after a certain amount of sessions, or at the end, or in the middle, or in the beginning, you can choose.

P: I mean it's really, I think that's also a bit how we do it in our department, where it's not necessarily like "you have to evaluate after this amount," you just do it naturally after like 10 sessions.

P: But maybe you also say, before the client goes on vacation, we're doing an evaluation, you kind of pick your own time.

P: Yeah.

I: So what concerns, if any, would you have about AI tracking patient progress?

P: I mean, I have so many concerns.

P: I don't like the idea of AI being really used in this field, and not because I think "oh my God, they're taking over my job," but mostly because the information is so sensitive and we're already doing so much to make sure that people don't even have access, let alone that it gets on a data server.

P: And a lot of the big AI companies are owned by US companies, and the rules that are there, or the lack of rules or limitations, it's just really dangerous to know that some client that comes into this Dutch healthcare system, that technically the US government has access to that.

P: That's everything that everybody has been dealing with the last couple of years in healthcare and privacy in healthcare.

P: It would just all go out the window.

P: That's a specific concern that I have.

P: And then apart from that, it's also like, psychology is not really a hard science, even though psychology really wants it to be a hard science.

P: So much happens in the room and observationally.

P: And again, I'm also like, is the AI that we have at the point where it can also track those things?

P: And I know that there's a lot of research happening and people are putting a lot of money into developing and investing in AI, but I'm not really sure if these are the things that they are also investing in.

P: Like I'm not really confident that these are also the things that these big investment companies are interested in developing further before they want to implement it on a wider scale.

P: Yeah.

P: Those are some concerns.

I: Yeah.

I: Okay.

I: Let's move on to the next question.

I: So this is about administrative and practical support.

I: How do you feel about AI generating drafts or reports?

I: Oh, actually, no.

I: I had a question, I remember now.

I: Medenda, right? Medenda?

P: No, it's Medendo.

I: Okay, Medendo.

I: Yeah, Medendo.

I: So do you know anything about their data privacy stuff?

I: And like how that works?

I: Because you said that's a concern for you?

P: Like, we asked about it a lot.

P: We as psychologists, or we as the company, we had a couple of meetings where we met the team from Medendo and we could ask a lot of questions.

P: And people were also curious about where their server is, and they are a Dutch company and it's all on a Dutch server.

P: And also the reason that they were chosen is because they were very upfront with where their data was stored and how they were dealing with it.

P: But they did say that it's a Dutch server, but like very originally, if you go back, it all still goes back to like a US company or something on a US site.

P: So yeah, it's safe, but also how safe is it?

I: Yeah.

I: Okay, then let's now move on.

I: Administrative and practical support.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: Yeah, I don't have any problems with that.

P: That sounds fine.

I: That sounds fine, that sounds good.

I: So follow up question, would you trust AI generated reports or would you need to review everything carefully?

P: Oh yeah, the second part obviously.

I: Yeah, we kind of already talked about it, right?

I: You use Medendo for transcripts and it makes reports and then you just review it, right?

I: How much time could this save you?

P: Honestly, Medendo, it shaves off hours.

P: Like it can take like four or five hours to really write the report.

P: And now it's like maybe an hour, an hour and a half, two hours to kind of read it all back, change a lot of stuff, write stuff, add things, edit, read it again.

P: I think it would take off like three hours or so.

I: And this is mostly in the intakes, because you said you don't really use it during the normal sessions.

P: Yeah, this is like, for example, the diagnostic process, the assessment part. And then for treatment I personally don't use it, but I know others do.

I: But do you feel like the use during normal sessions would also benefit from this time wise?

P: Yeah, yeah, obviously. Obviously if it's already written down that takes away the time.

P: I think it's a very personal thing.

P: There are some people who are very meticulous about creating reports and they really want it to be their own and they'll change a lot of stuff, and then you'll end up also spending 30 minutes on writing the report if Medendo did it, if you're also changing it and it takes up so much time.

P: There are other people who, it's not that they don't care, but they're kind of more content with the reports that Medendo is writing.

P: It would save them a lot of time.

I: Yeah.

I: Okay.

I: What concerns would you have about accuracy or liability?

P: Yeah, I mean, that's a big one, especially in terms of healthcare and AI.

P: But I also think that the AI that's been implemented in healthcare is very cautious about that as well.

P: So we're all quite alert when we look at Medendo reports to kind of see, how accurate is it? Is it being too suggestive?

P: And I feel like they're not going to really quickly implement AI that's going to decide specific results or is going to do diagnostics already.

P: I would say it's different in healthcare, and then mental health care is maybe something else.

P: And again, it's really not a hard science.

P: It's so suggestive.

I: And how about liability?

P: What would be an example of liability?

P: What do you mean by liability?

I: Well, the thing is that normally a psychologist makes everything, right?

I: So you're liable for things when things go wrong or if something is incorrect or whatever.

I: But then if you're using other tools, then,

P: Obviously, I think that would be a big concern.

P: But I still think that if you're the psychologist, the moment that you're a psychologist and you're coupled with a patient, if you write a report, whatever you do, you're still the person implementing AI, I absolutely think it's still your responsibility to check what the AI is doing.

P: If that's something you stand behind, if you also think it's accurate.

P: If you are not checking it and it's writing stuff down that's inaccurate or wrong, it's still your responsibility towards the client or patient.

I: Yeah.

I: Okay.

I: Makes sense.

I: Would this change how you document your work?

I: I mean, has it changed how you document your work?

P: Has it changed how I document my work?

P: Not really.

P: I think everybody's just adjusting to the AI doing the work that they were already doing in terms of writing the report.

P: But I'm not letting Medendo decide how I'm going to report stuff or do my reports.

I: Yeah.

I: So it's essentially just taking over some of your tasks, but still making it in a way where you would have created it, right?

P: Yeah.

I: Makes sense.

I: Okay, let's move on to the next question.

I: This is about psychoeducation and between session tools.

I: What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: Again, I'm thinking about the added benefit, like sure you could implement it, but what would it add?

P: Because like now we also have a lot of online modules that people can do after sessions or between sessions, and I can already tell patients hate that, they don't want to do it.

P: It's a chore for them to do, and I'm thinking, what would be the added benefit of AI?

P: Then I'm thinking more like the ChatGPT kind of thing, where it feels like there's a person that's like listening to you or talking back to you, and I've heard patients already talk about that, they do enjoy it, they like it, they also use it.

P: If you have to do a homework assignment for the sessions, to do it with a ChatGPT kind of chatbot, I do think that patients would enjoy that more.

P: If it could give some kind of motivation or some kind of reflection, or like if they're stuck, how should I continue with this homework assignment?

P: Like, we always tell patients that they can just contact us if they're stuck, but there's like a level of, they don't usually easily do it.

P: Maybe they feel a little bit of a burden to do that.

P: But I feel like if there was a chatbot, I think they would do it maybe more often.

I: Yeah.

I: So what if the AI provides information that contradicts your treatment approach?

P: Yeah, obviously I'm not gonna listen to it, no I'm kidding. Well, it's also not that isolated, like, because if I work on something then I also have a supervisory psychologist who's always seeing the patient with me, so I never see a patient on my own.

P: I think it matters more what that supervisory colleague psychologist would say, or what my team says, than what I would listen to the AI.

I: Yeah.

I: Yeah.

I: Okay.

I: Makes sense.

I: Would you worry about clients relying too much on AI between sessions?

P: I don't know.

P: I feel like this is a pretty big question.

P: There are certain patients of mine that I know talk a lot with AI or like with ChatGPT about their problems.

P: I have not yet encountered it becoming problematic or having an influence on the sessions that we have.

P: So at this point, in my own experience, I've not seen it go wrong.

P: But what does happen with ChatGPT, for example, or like chatbots, Claude as well, is that they're really not, well, obviously they're not people, they're chatbots.

P: So they give a really specific kind of reflection.

P: How should I explain this?

P: Sometimes people have problems in their lives and they have problems with other people in their lives.

P: And sometimes it's really helpful if you have another person in the room with you, like a psychologist, because the psychologist can then also feel, hey, what is this person doing in contact with others that's causing these problems?

P: And that can be quite a helpful tool to understand them better or to kind of see where their problems are coming from.

P: And this is something that a chatbot doesn't have access to yet.

P: And I feel like that's a sense of reflection that people tend to miss.

P: So what I often hear is that sometimes they'll be in a specific mood or they'll have specific fears and stuff.

P: And the chatbots can kind of seem to sympathize with them, but they can't always give an accurate reflection.

P: And they kind of just go along with whatever they're saying, but they can't pinpoint, maybe you're wrong about this, or maybe you're not seeing it correctly.

P: And if you ask ChatGPT to contradict everything you say, ChatGPT will do that, but I don't really think patients do it when they're talking about personal problems of theirs.

P: That's a specific thing that I feel like, okay, I don't know how the AI will account for that, that really the person who is seeing another person can kind of tell what's happening, which is not always in the verbal information.

I: Yeah.

P: Does that make sense?

I: Yeah, I think what you mean is that there is some way that chatbots respond where it kind of feels like confirmation bias, or like, yeah,

I: And sometimes it seems to lack an ability to reflect maybe harshly on someone, so maybe a psychologist would say, hey, maybe you have a part in this or like some problem that you created, right? And AI just doesn't do that that fast, or at least sometimes it seems like that.

P: Yeah, but it's also like very specifically, like OpenAI ChatGPT for example maybe is not as,

P: Not like instructed or programmed to do that often or very quickly.

P: Maybe another chatbot would do it.

P: I'm not sure.

P: And this makes sense because we talk about confirmation bias.

P: I have one patient who has told their chatbot to be harsh and to contradict them and not agree with them.

P: But I can tell she is very intelligent, and she's also doing really well.

P: And there are other patients that I have that aren't really doing well.

P: And they would never give that instruction to their chatbot.

P: So this vulnerable group of people is also not going to ask their chatbot to be harsh.

P: So that instruction, they wouldn't give it, I think, really quickly.

I: Yeah.

I: Yeah.

I: Like especially for people in the beginning of their progress, right?

I: If you're depressed or something and you need to change your thoughts, you don't have the idea to say, hey, try and make me think differently, right?

I: Yeah.

I: Okay.

I: How would you monitor what the AI is telling your clients?

P: I think, I mean, now also with chatbots you have the entire transcript, so I think if that was just accessible, yeah.

I: Yeah, okay.

I: Let's move on to the next question.

I: This is about risks and limitations.

I: What risks or concerns do you have about using AI in clinical work?

P: Well, the risks, obviously there's the data privacy risk that I think is really big.

P: And the second part would be more what I talked about before, where I think there is a very human element in therapy sessions.

P: It's not something that we need like food and water and sleep.

P: It's something that we've added on in a very, this is a way that we are dealing with problems in this really Western world, that we do therapy sessions.

P: But a big way that therapy has kind of developed, the idea of therapy and sessions and the way we do it, it's very much with the idea in mind that there's a person in the room with you who's listening to you, the human component of it all.

P: And I think that verbally a chatbot or AI could not even mimic it, but could take over that place.

P: And I think the verbal information and giving somebody a space where they feel like they can talk about their problems, or they have a feeling that they're less alone, I do think that AI can have a role in that.

P: You can already see that people do it, like there's a whole thing around AI therapy and AI companions and all that stuff.

P: So obviously it is mimicking a certain human element, but I don't think it can mimic it to the extent that allows for that part, that part of like, there's a person in the room and the person can kind of hold up a mirror to you or could tell you things because it's a person that's experiencing you.

P: That you can experience somebody being negative and you can kind of give that back to them and reflect that back to them in a way that somebody can't do if you're not really experiencing the other person.

P: And I think that's the human element that an AI chatbot, with the technology we now have, can't do.

P: And I do think you need it in order to have full progress, and for an effective therapy session, for effective therapy in general.

P: Yeah.

P: But also maybe in five years I'm wrong and the AI can do it.

P: And so what's the point anyway?

P: Yeah.

I: We'll see.

I: Okay.

I: So you talked about data privacy and data security.

I: Have you thought about issues like bias in AI systems?

I: We shortly mentioned it.

P: I feel like obviously there is going to be bias in the AI.

P: But maybe the reason I'm less concerned about that is because I'm already very distrustful of everything the AI says.

P: So I'm obviously checking everything it's saying before we implement it or put it in somebody's file.

P: So the way we're using AI right now, it hasn't taken over stuff to the point that we don't have our eyes on it.

P: Everything it's doing, we're still checking, we're still very much reviewing it.

P: So if there is bias, we will take it out.

P: Yeah.

I: Yeah.

I: What concerns do you have about transparency or explainability of AI decisions?

P: So in terms of how much the clients or patients know?

I: No, not really.

I: It's more about sometimes AI systems come up with something, but then you don't really know what it's based on or where it came from.

P: So if they come up with something that we should have access to, like the theory they have access to or a research paper that they're pulling something from.

P: Oh, that's interesting.

P: I've really never thought about that.

P: Well, I'm kind of thinking about when AI would come up with something on their own in terms of healthcare stuff.

P: If there are moments where we have AI that we're actively using in a creative manner and they're coming up with new things, I feel like obviously we should be able to track where they're getting the information from.

P: I think with ChatGPT you can also click on certain things and they can give you their sources.

P: If that would be similar to the way the AI is doing things, I think that would be a good way.

I: Yeah.

I: Any other limitations that you want to mention?

P: Well, limitations.

P: I don't know a lot about AI.

P: I don't know a lot about what is possible there.

P: All I know is that if there is a limitation I have in my head, I'm so sure that people will find a way to make that work in terms of AI.

P: It's not necessarily like the model itself.

P: Like I don't think the AI itself is bad or evil or should not be used or could not be effective.

P: I think in the model itself or in the technology itself, it can be very effective and can be really useful.

P: I just really think everything outside of it, like how people are implementing it, the people behind what decisions are being made, and what the investments are that are being made, that's where more of my concerns lie, my fears lie.

P: Because I just don't trust that, or have very little faith in that.

I: Yeah.

I: Okay.

I: Makes sense.

I: Let's move on to the next question.

I: This is about the client perspective and therapeutic alliance.

I: How do you think clients respond to AI being used in their treatment?

P: It really differs a lot.

P: Some people are very open to it because they don't understand it.

P: Other people are open to it because they're big technology nerds and they really like it.

P: And there are maybe like, let's say, 50% who don't want it, they don't like it, and they're quite distrustful of it.

P: And it's mostly also because they're very wary of where the information is going to.

I: Yeah.

I: Okay, makes sense.

I: So can you also score this? How do you think clients respond to AI being used in treatment, with a score of one being they don't want it to be used and ten being they want it to be used?

P: Four. Okay, I'm gonna say three, they really don't like it.

I: Okay, yeah.

I: How might this affect the therapeutic alliance if we were to use AI?

P: Well, I mean, I do think it's like a breach of the relationship you have with your client if you're using something that they're quite wary towards.

P: Should I, do you want me to score this?

I: No, this is the open question again.

P: Okay, okay.

P: Well, yeah, I do think it will influence it.

P: Because obviously, it's already a big step for people to come in a room and meet somebody they've never met.

P: And if they're also using a program that they don't like, you obviously become this distrustful figure.

P: So I would say it impacts it negatively.

I: Yeah.

I: Do you think clients will be more or less engaged with AI supported treatment?

P: I think it differs, because I know that when we talk about Medendo, for example, and how we're going to use it, people have a lot of questions.

P: They want to know where it's going and they don't really like it.

P: That's usually the main response that we get.

P: But I also know that these are sometimes the same people that do talk to ChatGPT and use it.

P: So it does also seem to be like this question of AI is kind of new and people are just hearing about it sometimes for the first time.

P: They don't really know what it is or how it's being used.

P: There's a lot of confusion around it.

P: So I think sometimes, I forgot your question.

P: What was the question again?

I: Do you think clients would be more or less engaged with AI supported treatment?

P: Like, I feel like if we did a chatbot between the sessions, they'd love it.

P: But then they're really weird with Medendo again.

P: So I think it's more in the confusion, like if they understand what the AI is really doing, then they'd be more open to it.

P: Yeah.

I: Okay, last question.

I: Professional identity, autonomy and future conditions.

I: Which aspects of your work should remain strictly human?

I: And what would you need to feel confident about using AI safely in practice?

P: Well, all of it.

P: What should stay strictly human?

P: I mean,

I: You already mentioned a lot of things, but you can summarize a bit.

P: Obviously I think the sessions and the human components, stuff like that.

P: But I'm doubting myself because I'm thinking, how different is it what I'm doing versus what AI is doing, and stuff like BetterHelp?

P: They're saying it's a person behind BetterHelp, but what if it's an AI model?

P: And then it's like, okay, why should it then strictly remain human?

P: I don't know.

P: I feel personally like I think it's important to have a human person in the room who you're doing the sessions with.

P: I think that's a benefit that AI does not have.

P: Yeah.

I: Yeah.

I: So why do you think these aspects need to stay human?

P: Why?

P: I think it's more accurate when it's a person compared to the technology we now have when it's AI.

I: Yeah.

I: And what would you need to feel confident about using AI safely in practice?

I: That's just like all of the risks probably that you want to minimize.

P: The law needs to be updated.

P: It would make me feel more safe if I knew there were actual laws put in place that were being kept up with the development of AI.

P: That would make me feel more safe.

I: Yeah.

I: Okay, anything more to add?

P: Yeah, like, are you making an AI model?

P: What are you doing?

I: I'm not making an AI model.

P: Well, anything to add?

P: I don't know.

P: I have nothing to add.

P: I think this is an interesting time and place where we are trying a lot and we don't really know.

P: I think the question that needs to be asked is the added benefit.

P: What is constantly going to be the added benefit of using AI?

P: And also not the added benefit in terms of, okay, how are we going to make more money out of it, because it's maybe attracting a lot of clients.

P: But does this in the long run mean that clients are helped more? Does it make their complaints go away faster?

P: Does it help with the suffering that they have?

P: Those are, I think, important things to keep in mind when we talk about the added benefit of AI.

P: Yeah.

P: Because you could take every person away and just do AI.

P: That's cheaper.

P: That's so much cheaper.

P: But yeah.

P: That's my closing sentence.

I: Done.

I: Then I'm going to stop the recording.